

CONTRACT EMERGENCY RESIDENTIAL SERVICES BEDS

1. **PURPOSE**

- 1.1. The Department of Veterans Affairs Palo Alto Health Care System (VAPAHCS) located at 795 Willow Rd. Menlo Park, CA 94025 requires a contractor to provide services as part of its Community Based Health Care for Homeless Veterans (HCHV) program. The goal of the HCHV program is to remove homeless Veterans from the street or habitation unfit for Veterans and place them in community-based, residential environments with sufficient supportive services to meet their needs and ultimately facilitate the improvement of their housing situation. This contract represents a Firm Fixed-Priced, Indefinite Delivery-Indefinite Quantity contract. Costs not incorporated into the Contractor's price will NOT be reimbursed by the Government. This contract is specifically for the geriatric population which has additional requirements as outlined below.

2. **BACKGROUND**

- 2.1. Through the HCHV program, VA provides supportive services to Veterans and facilitates their access to a broad range of medical, mental health, and rehabilitative services. The purpose of this solicitation is to obtain offers from Contractors in **Monterey and Santa Cruz Counties** who can provide housing, and supportive services to geriatric Homeless Veterans that, in addition to community-based, the Contracted Emergency Residential Services (CERS) facilities will be stand-alone and have a home like environment. Geriatric is defined as a veteran who is 50 years of age or older. However, exceptions to this age requirement of admission can be made on a case-by-case basis. These facilities will offer a safe and secure environment that supports the goal of recovery from homelessness. The Contractor will be required to provide a low-barrier therapeutic and rehabilitative milieu and attendant services targeting the underlying factors contributing to homelessness within the geriatric population. The Contractor will not be required to provide detoxification or other hospital level or licensed care treatment.

3. **SERVICES TO BE PROVIDED**

- 3.1. The Contractor shall provide all labor, supervision, housing, material and supplies necessary to provide emergency residential treatment, rehabilitative and supportive services. Services will be provided on-site at the Contractor's facility, in accordance with all terms and conditions, provisions and requirements listed herein. The prices provided in the Price Schedule shall be inclusive of all "basic services" as may be necessary in the treatment of the Veteran. "Basic Services" shall be defined in the Performance Work Statement.

- 3.1.1. **Residential Room and Board**: Room and Board shall be accessible to the Veteran 7 days a week and 24 hours per day.

- 3.1.1.1. One single bed and personal linens (towels, pillows, blankets, and bed sheets, etc.)

- 3.1.1.2. Furnishings such as a dresser, storage locker and lock or designated locked secured space. Storage space will accommodate two bags of belongings and all prescribed medical equipment. The Contractor will allow Veteran to store personal belongings for at least 72 hours after formal HCHV discharge. Should

a Veteran abandon his/her belongings, the contractor will make three documented attempts to contact the Veteran before disposing Veterans items. At least three nutritious meals, 7 days a week will be provided for the Veteran. At least 50% of the beds will be wheelchair accessible and will have access to ADA accessible entryways and restrooms.

3.1.2. **Dietetic Services:** Contractor will provide three meals a day and snacks of nourishing quality (e.g., fruits, vegetables, protein sources, etc.), between meals and bedtime for those requiring or desiring additional food, even when it is not medically indicated. There will not be more than a 14-hour span between evening meal and breakfast of the following day. Contractor will provide alternative meals for Veterans with dietary restrictions if medically indicated (e.g., diabetic, renal, and soft mechanical diets) and reasonable accommodation for Veterans with cultural/religious preferences around food (e.g., Kosher, Halal, etc.). Food shall be prepared, served, and stored under sanitary conditions. The facility shall provide storage space in an onsite refrigerator for Veterans to store and freely access personal food. The facility shall establish and maintain sanitary procedures for washing dishes, cleaning equipment and work areas, and disposing of waste. In addition, for locations where food is provided via food pantry, Veterans will have access to the pantry at least three times a day to access foods. All foods shall be stored and maintained in sanitary conditions following all health and safety regulations. Veterans will be offered food of nourishing quality and variety (fresh fruits, vegetables, grains, breads, and proteins).

3.1.3. **Laundry Facilities:** Laundry facilities and necessary detergent free of charge shall be available for residents to do their own laundry or to have laundry done at minimum one time per week. Should a Veteran have difficulty doing his or her own laundry, the facility will provide guidance. Should a Veteran refuse to wash his or her clothes, this will not be grounds for discharge but rather be addressed as a therapeutic opportunity.

3.1.4. **Case Management Services:** This contract requires that the Contractor will maintain a minimum staffing ratio of one dedicated, full time, on-site medical case manager per 12 Veterans. (For further information on staffing requirements please see Section 6: Personnel) Exceptions may be granted in writing by the VA. Staff is defined as a paid professional and does not include interns. The VA requires that case managers working with Veterans do not engage in any outside duties that do not pertain to the HCHV program which includes, case management of or group facilitation of non-Veteran populations unless a waiver has been granted by the VA Liaison (or Designee). Each VA Liaison (or Designee) will have no more than two POC's per program. Additionally, the Case Manager shall provide the following:

3.1.4.1. Intake Packets: All Veterans must receive an intake packet within 72 business hours which includes, at minimum, the following information:

1. Description of the Contractors services
2. Grievance Policy
3. Emergency procedures
4. Patient rights and Reasonable Accommodation

3.1.4.2. A thorough assessment of Veteran's psychosocial needs and written Individualized Care Plan (ICP) will be developed within 72 business hours of admission for each Veteran. Information from the assessment will be integrated into the Veterans' Individual Care Plan. Care Plans will also include

an assessment of possible crisis for the Veteran such as harm to self, verbal abuse or acts of violence or any other medical or psychiatric crisis. Staff will take proactive measures to prevent these instances to avert crisis to keep these incidents at a minimum. Contractors will be proactive to ensure staff are trained in de-escalation techniques.

- 3.1.4.3. Case Manager will provide structured individual case management, at minimum, weekly including counseling on self-care skills, adaptive coping skills, financial planning, permanent housing search, written care plan, referral for financial benefits.
- 3.1.4.4. **For Santa Clara County locations, in addition to their standard documentation, the case manager(s) will also be expected to update HMIS on all admissions, discharges and enter at least one transaction in HMIS at per month.**
- 3.1.4.5. Collaboration and coordination with VA program staff, as needed, will include coordination of supportive psychosocial services. Coordinated efforts must be made with the Liaison around medical, mental health, admission and discharge needs. Contractor is expected to coordinate all appointments including, but not limited to chemotherapy appointments, dialysis appointments, and wound care appointments. Contractors will be expected to accompany Veterans to their appointments when medically, cognitively, or psychiatrically indicated per the liaison and/or the contractor's clinical judgement. Contractor staff will make active effort to establish relationships with key stakeholders, providers, and partners in the VA Health Care and Benefits Administration Systems to work closely and collaboratively with the interdisciplinary treatment team.
- 3.1.4.6. Ongoing Care Plan Coordination will be expected between the Case Manager and VA Liaison (or Designee). The Case Manager will be expected to coordinate with the VA Liaison (or Designee) to ensure Veterans' goals, as expressed in both the agency and VA Liaison's (or Designee's) intake, are reflected in the ICP's.
- 3.1.4.7. Every 60 Days the Case Managers will coordinate a meeting between the Veteran and the VA Liaison (or Designee) for bi-monthly ICP update meetings. The Care Plan meeting may be used by the Liaison (or Designee) as the main factor to determine if any Veteran extension requests may be approved or denied.
- 3.1.4.8. The Case Manager will be expected to meet with the VA Liaison (or Designee) for case management update meetings every 30 days. All Contractors will provide proper documentation verifying services and case management efforts by all team members including, but not limited to housing, benefit and employment specialists, and program management staff. There will be an expectation that notes are written professionally in a format that utilizes the clear settings of goals and documents progress towards those goals (e.g., SNAP, SOAP, or SMART notes).
- 3.1.4.9. Special attention will be made to address High Suicide Risk (HSR) Veterans as identified by the VA Liaison (or Designee), VA Mental Health Staff and/or the contracted Case Manager. Contractor will avail themselves to be contacted by the suicide prevention coordinator immediately should a Veteran miss an appointment and his/her safety must be verified. On a weekly basis the contracted case manager will be required to update the VA Liaison (or Designee) on Veteran progress and/or safety concerns for HSR Veterans. Contracted case manager will review the HSR Safety Plans with the Veteran

on a bi-weekly basis. HSR Safety plan reviews and a copy of the Veteran's Safety Plan will be documented in the chart. All Veterans will be referred to VA for Primary Care and Mental Health appointments and Contractors will support Veterans in making initial and subsequent appointments. Efforts will be made for Veterans who are identified by Liaison to be highly acute. Efforts will be documented in the Veterans ICP and reviewed by the VA Liaison (or Designee), at least monthly.

- 3.1.4.10. If the Contractor determines a Veteran who has not been identified as an HSR Veteran to have expressed suicidal ideation, the Contractor will be expected to follow their agency and VA incident and safety protocols. If the identified Veteran does not have a safety plan, the agency will create an agency safety plan with the Veteran and make increased efforts to connect the Veteran to mental health services. The agency safety plan will be forwarded to the VA Liaison (or Designee) as well as information on all efforts to connect Veteran to mental health services. As above, all procedures applied to HSR Veterans will then be followed for the non-flagged HSR Veteran until the Veteran can be evaluated by a MH provider and/or Suicide Prevention Coordinator.
- 3.1.4.11. Contractors must be in close communication with the liaison for any missing Veterans due to the high acuity of the target population. The contractor must file missing person's report should the Veteran be considered at risk.
- 3.1.4.12. Contractor staff will treat liaisons with respect and will work with assigned liaisons to resolve issues around quality of care and contract compliance with respect.
- 3.1.4.13. Medical Case Management Services: Provides support to patients in maintaining health and wellness in the Geriatric program. Assists veterans with scheduling medical and mental health appointments, provides customized education for medical diagnosis and medication management, and arranges appropriate transportation to and from all appointments. Veteran income cannot be used as a barrier to arranging or providing transportation. Contractor makes referrals to community-based organizations when health and/or psychosocial condition(s) indicate need for appropriate referral(s) and makes sure all medications prescribed are reconciled to prevent duplication of orders among providers. Promotes clear communication amongst treating providers by ensuring awareness regarding veteran's medical needs and prescribed medication. Provides coordination of care by meeting with veterans and medical staff to determine goals and recovery targets for identified conditions. Assist veterans with medication management by monitoring Veterans' compliance issues and providing extensive medication education to improve compliance. Teaches appropriate medical interventions regarding wound care, self-care, and management of medical diagnosis. Contractor is expected to offer cues to Veterans to facilitate self-care. Staff giving this education will be qualified professionals to offer this service and education to Veterans.
- 3.1.4.14. Permanent Housing Search: Programs are required to provide Veterans with direct and ongoing assistance in achieving permanent housing. Plans for Veterans' transition to permanent housing placements must be clearly reflected in each ICP and in weekly case management notes. As part of this plan, Veterans' housing history and needs must be assessed through a formal and thorough Housing Assessment completed within 72 business hours of program admission. Housing Assessments must identify Veterans' housing history, strengths (i.e., positive prior rental history), and barriers (i.e., Unlawful Detainers). Should Veteran refuse to engage with program staff regarding plans

for permanent housing transition, motivational interviewing and other therapeutic techniques will be used to address Veteran ambivalence. All efforts to engage Veteran regarding housing plans, options, and resources shall be clearly documented and include any therapeutic techniques utilized. Contractor staff must provide direct assistance to Veterans in developing permanent housing plans and accessing appropriate housing resources. These services shall be provided weekly through one-to-one case management. Examples of expected housing services include: 1) assistance obtaining and reviewing Veteran's free credit report, identifying housing strengths and/or barriers associated with current credit status; 2) Creating a Tenant Portfolio with Veteran that contains all relevant documents required in the rental process, including a completed sample rental application for Veteran, income verification documents, identification, applicable subsidies, etc.; 3) Reviewing affordable housing rental options and Permanent Supportive Housing (PSH) program offerings, and assisting Veterans with making appropriate rental inquiries and submitting completed rental applications; 4) Conducting regular and individualized housing advocacy efforts, including transportation, assistance scheduling, meeting, and communicating with property managers and landlords, and engage in troubleshooting where rental barriers or denials occur in the housing search process 5) exploring housing option in low-rent areas 6) exploring appropriate level senior care facilities. All housing efforts by Contractor shall be documented clearly **in case notes** and presented to VA Liaison (or Designee) upon request. Mere communication with Veterans regarding housing, without subsequent Contractor staff action and follow-up, shall not on its own meet the above requirements. If housing placements or inpatient residential treatments have been approved and acquired, the support and case management around maintaining housing, future housing options, connecting to local resources, discussions on how to maintain/pay for housing over long term, adult day health referrals and other community long-term supports may be counted as a housing resource.

- 3.1.4.15. Financial Planning and Employment: Referral to and follow up on all potential financial resources the Veteran may be eligible for (e.g., SSI, SSDI, Food Stamps, NSC pension, GA, Medi-Cal, etc.). Case Managers will document in the case notes all structured activities that support Veteran in developing a short and long-term plan to understand and address their current financial situation and how to improve it. At minimum two financial planning activities appropriate to the Veterans individual circumstance must be documented monthly. All Veterans will be assessed for employment within 72 hours of admission. Veterans who report a desire to work will be provided referrals to all relevant employment opportunities the Veteran is eligible for and interested in and/or structured activities that support Veteran in applying for employment as appropriate based on needs of Veteran as identified in the assessment. Employment referrals and activities may be counted as a financial resource. The Case managers will document the assessment and all referrals and supportive activities in the case notes. Each Veteran must apply for Medi-

Cal and Medi-Care if they do not have this benefit.

- 3.1.4.16. Group Activities: Structured group activities, as appropriate, shall occur two times per month – examples include group therapy, life skills training, social skills training, financial workshops, Alcoholics Anonymous, Narcotics Anonymous, vocational counseling, and physical activities as appropriate. For the purposes of this contract “House Meetings” and VA Housing Resource Groups may not be counted as a structured group activity should have a therapeutic value and facilitation shall be the responsibility of the Contractor. Should a Veteran have mental health, medical, family and/or employment obligations which directly conflicts with this requirement alternate group activities will be arranged and/or the Contractor will clearly document in their case notes their efforts to accommodate the Veteran. Unwillingness to participate in these groups will not be considered grounds for program discharge.
- 3.1.4.17. Case manager will make referral to all relevant legal services which would further the Veteran’s efforts to get housed. Case managers will document all activities and education that support a Veteran’s legal needs.
- 3.1.4.18. Discharge Planning: Securing permanent housing will be the primary discharge goal for every Veteran, and resources will be coordinated for discharge to a successful community placement throughout the duration of the Veteran’s stay.
 - 3.1.4.16.1. All Veterans must have a discharge plan within one week of admission which integrates items included in earlier sections of this document.
 - 3.1.4.16.2. Veterans must also be in active search and/or apply for independent housing within the first month of admission. Compliance with all VA regulations regarding discharges and timeliness of reporting discharges is required by Contractor. Paperwork required for discharging a Veteran from any HCHV program is required via fax or secure email within 24 business hours from known discharge.
 - 3.1.4.16.3. A negative discharge for undesirable behavior is a committee decision that ***must*** include the VA Liaison ***prior*** to discharge. All Veterans scheduled for discharge based on behavior must meet with the treatment team and work with team to determine most therapeutic option for Veteran. *This does not include violations for safety reasons.* Please note: Profanity does not in and of itself constitute abuse and shall not exclusively be considered grounds for discharge. All actual or threatened violence *will* be grounds for discharge. Veterans may be discharged for safety reasons at any time. Safety concerns must be clear and obvious to both contractor and liaison. Parameters around safety violations and/ what constitutes reasonable threat must be established with liaison. All negative discharges will be subject to a full team debriefing, including VA Liaison, to look for opportunities missed to intervene sooner. All discharges are subject to the Contractors grievance procedures and must allow clients the opportunity to be represented by the VA Liaison in the grievance process. Reasonable efforts must be made to coordinate with the Liaison to schedule an appeal.

In the case of no reasonable alternative housing arrangement, a target discharge date may be established by agency, no less than 7 calendar days of decision to discharge, to afford Veteran an opportunity to utilize VA Liaison advocacy.

3.1.4.16.4. Lastly, all TQI measures must be met to receive the highest past performance rating. These include the following:

- HCHV1 (DC to Ind. Housing) - > 55%
- HCHV2 (DC w/ Neg. Exits) - 20%
- Bed Occupancy > 85%

3.1.4.19. Extensions- It is understood that the Contractor will not be paid for care provided to a referred Veteran beyond the period authorized, unless an extension of the authorization is provided in writing by the VA. The initial stay for a Veteran is expected to be no longer than 60 days. Any extension of the stay after 60 days must be authorized by the VA Homeless Program Coordinator or Designee, if there is clear clinical indication and availability of funds. Only extraordinary circumstances will be considered to extend service periods up to and more than 6 months for individual Veterans and these must be authorized by the Medical Center Director or Designee.

3.1.5. **Medication Management:** Medications and narcotics shall be properly stored, controlled, issued, and recorded in compliance with physician's orders. For Geriatric Contracts it is recommended that facilities utilize a Medication Management system for Veteran medications. (Copies of Inspection Packet requirements relating to medication management are available upon request).

3.1.5.1. Contractor shall establish procedures for insuring Veteran's confidentiality in the storage of and keeping of records concerning medication.

3.1.5.2. Medication Assisted Treatment (MAT) cannot be used to rule out Veteran's participation in an HCHV Program. This includes prescribed use of Vicodin, morphine, methadone, oxygen, etc.

3.1.5.3. Reasonable accommodation for individuals in MAT is required provided the requested accommodation does not require major financial or administrative commitments that would be considered an "undue burden." Examples of reasonable accommodations include:

1. Arranging for the individual to take medication at their clinic, physician's office, or another off-site location – when consistent with the individual's treatment plan.
2. Storing an individual's MAT medication in a lock box in the program and having the individual be personally responsible for it.
3. Arranging to have the housing facility keep MAT medications in a locked cabinet For Facilities that are providing Medication Management, there must also be staff identified for after hours and on weekends who are trained in medication management protocol who can provide medication to the Veterans as needed when the medical case manager is not on duty. This coverage must be and the certificate(s) of medication management training and the times/days and hours of the person(s) covering will be provided to the VA

Liaison to keep on file. (Copies of Inspection Packet requirements relating to medication management are available upon request).

4. Arranging to have refrigerated medications stored in appropriate locked location which is only accessible to the Veteran or trained designee. (Copies of Inspection Packet requirements relating to medication management are available upon request).

3.1.6. **Grievance Procedures:** Programs must have an internal grievance process that Veterans can use to resolve conflicts within the program. Programs must have written policies and procedures for resolving grievances, including a statement regarding the client's right to request reasonable accommodation, and must post them in a place conspicuous and be accessible to clients. In addition, each client shall receive a copy of the grievance policies and procedures, upon intake and upon receiving a warning or discharge notice.

3.1.7. **Timeliness of Response:** Contractor is expected to have a dedicated phone line for VA inquiries and Veteran self-referrals. Return calls from the program are expected within 4 hours during business hours Monday through Friday. Calls, referrals, emails, or other correspondence from, community agencies, VA staff, Veterans or VA Liaisons (or Designees), are also expected to be addressed within 4 business hours Monday through Friday. Admission determinations must be made within 24 hours of referral.

3.1.8. **Critical Incident Reporting:** All critical incidents will be reported within 24 hours. This includes the following:

- Falls
- Assault (to Veteran or Staff)
- Elderly/Dependent Adult Abuse or Neglect
- Sexual Assault
- Fire (Veteran Involved)
- Medical/Psychiatric Emergency (911 Calls)
- Hospitalization
- Suicide or Suicide Attempt
- Homicide
- Death
- Infectious Control (TB COVID, etc.)
- Active Substance Abuse on contractor property
- Observation/ Possession of Weapons
- Use of "cooling off" option within facility
- Grave Disability

3.1.9. **Therapeutic and Rehabilitative Services:**

3.1.9.1. Milieu: The agency will cultivate a milieu of trust and safety. This includes a tolerance for relapses, regression, and personal unpredictability. In cases of documented verbal escalation with other program participants and/or Contractor staff, Contractor is permitted to request Veteran voluntarily relocate to "cooling off" area, as long as the area is within the physical boundaries of the Contractor's HCHV facility. Veteran will be monitored in a clinically appropriate manner by Contractor's staff, during any time within the aforementioned area. As a part of the engagement strategy, Veterans will be invited to participate in the activities of everyday life, including meals, chores,

recreation activities, and socializing. These activities must maintain a therapeutic significance. Such opportunities will be offered, not required nor demanded and modeled by staff. Veteran's resistance to participate in these activities will not be grounds for discharge. A Harm Reduction Model will be utilized. Fidelity to this model may be assessed at any time by the liaison.

3.1.9.2. Geriatric Contracts: Contractors will be expected to assist with light self-care including, but not limited to, offering adult briefs, and providing daily cues for hygiene, self-care, medication, and eating. It will be understood that the target population in this contract are going to need additional assistance in completing activities of daily living. Contractor will also be expected to accommodate Geriatric Veterans with mild cognitive impairment, severe mental illness and substance use disorders. Contractor, as clinically indicated, will be expected to facilitate daily case management meetings, coordination with family, ombudsman and board and care facilitators, hospitals, substance use treatment facilities, and medical providers. Contractor is expected to assist Veteran in applying for Caregiver, HHA and IHSS services, Aid and Attendance and other benefits and services. Contractor will be expected to have increased understanding of aging and dementia needs. While the contractor is not expected to offer licensed level care, the contractor will support Veterans who show a decline in daily functioning. As such, the administrator for this agency must be RCFE administrator certified.

3.1.9.3. Recovery: Support for a healthy lifestyle provided in an environment conducive to social interaction and the fullest development of the resident's rehabilitative potential. Assistance to gain and to apply knowledge of the recovery process in an environment supportive of recovery models including a focus on Harm Reduction rather than strict abstinence and supportive of a Housing First approach. Pursuant to these principles, drug testing may not be used to discharge or discipline a Veteran but may be used as a clinical intervention to modify behavior or assess behavioral issues.

3.1.10. **Transportation:** The Contractor shall assist the Veterans with round trip transportation to scheduled meetings, medical appointments, and hospital visits within normal business hours.

3.1.10.1. The Contractor will be expected to help the Veteran access public transportation, including providing information and instructions necessary to enable Veterans to utilize public transportation.

3.1.10.2. If VA staff determines that adequate public transportation is not available or appropriate for a Veteran, the Contractor shall provide alternative transport by car or taxi.

3.1.10.3. Contractor will also be expected to accompany Veteran to appointment to ensure the Veteran meets the provider should the Veteran need this added assistance.

3.1.11. **Occupancy:** It is expected that Contractor beds will be maintained at an average minimum of an 85% occupancy rate but at a preferred rate of 90% as calculated based upon number of bed nights available per quarter. After hours and weekend admission will be expected. If this rate is not consistently attained a Corrective Action Plan (CAP) will be developed by the Contractor as part of the QASP and/or process improvement plan. Adjustments in contract funding may occur through

modifications if occupancy is not maintained. Failure to maintain year to date occupancy rate at or above 85% may be grounds for immediate termination for convenience. Contractors must develop relationship with key referral sources in order to maintain referrals. All beds must be filled within 24 hours if there is a waiting list.

3.1.12. **Outreach:** Contractor will be responsible to engage in outreach in the community at least two times per month. Outreach activities may also include collaboration with the VAPAHCS providers for referrals. In addition, clear documentation for outreach activities must be made available to Liaison upon request during the QASP review. Contractors will maintain occupancy by engaging in outreach services and will be knowledgeable about outreach best practices generally accepted in the community.

3.1.13. **Documentation:** The Contractor shall provide treatment and discharge planning reflecting a team assessment of health, social and vocational needs and the involvement of the Veteran, the VA staff and appropriate community resources in resolving problems and setting goals. An individual case record will be created for each referred Veteran. Case records shall be maintained in security and confidence as required by the Confidentiality of Alcohol and Drug Abuse Patient Records (42 CFR part 2) and the Confidentiality of Certain Medical Records (38 USC 7332). Case records and data normally maintained and included in a medical record as a function of compliance with State or community licensing standards will be made available on a need-to-know basis to appropriate Department of Veterans Affairs staff members involved with the treatment program of the Veterans concerned. The Contractor shall comply with applicable requirements of the Confidentiality of Alcohol and Drug Abuse Patient Records (42 CFR Part II) and the Confidentiality of Certain Medical Records (38 USC 7332). The files shall include:

- Reasons for referral
- All essential identifying data relevant to the resident and his/her family including a socio-cultural assessment, weekly progress reports or notes, and documentation of any case management interventions or patient care conferences
- For sites who implement Medication Management/Monitoring- Copies of any medical prescriptions issued by physicians, including orders, if any, for medications to be taken. Contractor must be in compliance with HCHV inspection requirements
- Case management notes written in a professional manner (i.e., SMART, SOAP, SNAP) will include the following: A written and thorough Housing Needs Assessment, A written, standardized Employability Assessment, A Financial Plan, A Discharge Plan, and an Individual Care Plan with attendant goals and documented activity indicating Veteran and Case Manager is actively working on identified goals.
- QASP Compliance Tool and all attendant supporting documentation demonstrating Contractor compliance with Contract
- Final summaries on each resident who leaves the program, to include reasons for leaving, the resident's future plans, and follow-up locator information.
- Extension approval if relevant
- Waivers as appropriate

The Contractor shall comply with the principles listed in 38 CFR 17.707(b) to provide housing and supportive services in a manner that is free from religious discrimination.

3.1.14. **Rules, Policies and Procedures:** The Contractor shall have reasonable rules governing day-to-day life and activities in the facility. The purpose of these rules is to promote transition, safety, cleanliness, and dignity. Rules should not set up residents for failure and should be presented in a manner that is least intrusive and punitive. Infraction of rules are to be seen as a therapeutic opportunity, not responded to punitively. Should any rule prove to be a barrier to service, then the rule must be reviewed with the liaison and possibly changed to ensure continuous engagement and accessibility to the program. Response to rule infractions should be approached with leniency and engagement with further enforcement of rules as appropriate to the Veteran's engagement in the program. The focus of the rules is to be around the safety of Veterans residing at the facility. Rules regarding entry and exit of the facility, rules of substance use, chores, engagement in treatment, and general rule compliance, shall be as flexible as is necessary to carry out the functions of this contract and to maximize Veteran engagement. Rules traditionally applied in the shelter and transitional housing contexts may not be appropriate to the objectives stated here. Any rule or policy that proves problematic to the active, effective, and ongoing engagement of the Veteran shall be addressed by the VA and program management team, subject to immediate review and repeal upon the request of the VA Liaison. Examples of rules that may not be appropriate to this setting are breathalyzing upon intake, enforcing chores as mandatory, mandatory sobriety, mandatory meetings (house meetings, AA, NA, SUD treatment meetings). *Veterans will be provided a copy of the rules at intake and/or upon Veterans' request.* In addition, Contractor will post the rules in a location readily accessible to clients and visitors. These rules must include detailed Patient Rights and the procedures that the Contractor has in place to protect the Veterans rights and dignity. Veterans must be permitted to exercise these rights without fear of reprisal. If requested, Contractors must reasonably accommodate Veterans whose compliance with program rules is limited by the Veterans' physical or mental disabilities, in accordance with the Americans with Disabilities Act, the Federal Fair Housing Amendments Act, Section 504 of the Rehabilitation Act, including those requirements covering reasonable accommodations for disabilities and the use of assistance and companion animals, and the California Fair Employment and Housing Act of 1973 and all other applicable State or Federal laws. Contractor must equally apply all rules, policies, and procedures to Veterans, unless a Veteran has asked for a reasonable accommodation due to his/her disability.

4. **REFERRALS, ADMISSIONS, ELIGIBILITY, AND DISCHARGE**

- 4.1. **Referrals and Admissions:** Contractor is not permitted to require written applications and in person interviews from referral sources and/or Veteran as condition of admission to the program unless pre-approval has been obtained from the VA Liaison for special populations. For the Medical Respite and Geriatric programs, if there is an approved written application and this application is causing a delay in admission, the contractor is expected to reach out via phone to the referring party and/or Veteran and allow for any pending needed information to be relayed over the phone. Contractor may verify this information with the VA Liaison. Admission determinations must be made within 24 hours of referral. Liaisons may disapprove an admission based on lack of acuity for a Veteran at any time.
- 4.2. Liaisons may disapprove an admission based on lack of acuity for a Veteran at any time.
 - 4.2.1. HCHV contracts must have their own, separate interest list that cannot be combined with any other programs. Contractor must utilize a transparent admission process

that includes written eligibility criteria that is fair, objective, and compliant with applicable State and Federal laws.

4.2.2. The contractor must be able to accept potential admissions from select VA POCs after hours. These POCs will be clarified with the VA liaison.

4.2.3. Contractor will provide verbal and written notification that is readily accessible to all individuals including the VA liaison, upon request, of reasons for non-acceptance. Notification of reasons for non-acceptance is not required to be given to those denied service/housing because of bed unavailability. The eligibility information must be made available to clients at intake and staff must provide answers to questions about the admission criteria and process.

4.2.4. Contractors must have formal appeal procedures through which clients may appeal unfavorable admission or eligibility decisions and a copy of the policy for appeals is to be given to the Veteran at screening or intake. For highly symptomatic Veterans, a waiver for admission and intake procedures can be requested VA Liaison (or Designee).

4.2.5. Contractor cannot ban VA staff from the shelters if they are providing coordination, case management or care for Veterans who are enrolled in the VA programs. If there are conflicts with VA staff. Agencies must bring any issues with VA staff to the supervisor. The VA will make all efforts to work with agency staff on coming to a mutual agreement which will respect the agency and minimize care coordination with the Veteran. Admission process is subject to liaison review and adjustment at any time to ensure accessibility for Veterans.

4.3. **Eligibility:** The VA is responsible for determining eligibility of Veterans *prior* to acceptance by the Contractor for services.

4.3.1. The Contractor is responsible for requesting eligibility and written pre-approval verification from the VA Liaison (or Designee) (hard copy, fax or secure e-mail are acceptable) *prior* to admission and/or billing for services for each Veteran referred under this contract. If there is an urgent need to admit a Veteran and VA Staff is not available to provide a written approval in a timely manner a verbal approval is acceptable. Any admissions that occur without written/verbal approval must be verified by VA Staff in order for the Contractor to receive payment for time spent prior to approval (e.g., admission after hours or during the weekend). Written documentation of eligibility verification, signed by an authorized VA Staff, shall be obtained by the Contractor as soon as possible for each Veteran referred for services under the contract for inclusion in the Veterans program file.

4.3.2. A list of authorized VA Staff for the contract shall be provided to the Contractor upon award of the contract. VA Staff may be added or deleted from the list during the term of the contract at the discretion of VA Contracting Officer. The Contractor shall be provided an updated list of authorized VA Staff whenever such changes are made.

4.3.3. It is understood that the Contractor will not be paid for care provided to a referred Veteran beyond the period authorized, unless an extension of the authorization is provided in writing by the VA.

4.3.4. It is understood that the type of Veterans to be cared for under this contract will require care and treatment services over and above the level of basic room and board. It is also understood that Veterans may need more intensive case management than the general population. The level of intensity of services required will be part of an ongoing conversation with the Liaison.

4.3.5. To be eligible for placement in emergency contract beds, all Veterans must be homeless and be eligible and registered for VA healthcare services. Special

population contracts are intended for Veterans with special needs. The Geriatric contract is intended for Veterans over 50 however the liaison may grant admission to Veterans under 50 based off needs and acuity. Contractors will be expected to provide housing and or services to other special Veteran populations, such as but not limited to the following: medically compromised Veterans, Veterans with Cognitive disorders, Veterans with sex offender status, seriously mentally ill Veterans, Veterans who have abused drugs or alcohol for many years, and/or Veterans who have been involved with the legal system.

Denial of Services: Unless determined clinically appropriate by the VA liaison, Veterans cannot be banned or denied entry to HCHV Contracted Residential Services based solely upon length of current abstinence from alcohol or non-prescribed controlled substances, active use, the number of previous treatment episodes, the time interval since the last program entry, the use of prescribed controlled substances, disability, income, gender, or legal history. This includes a Veterans status of being a victim of Domestic Violence and as such cannot be considered during the screening process. The screening process must consider each of these special circumstances and determine whether the program can meet the individual Veteran's needs while maintaining the program's safety, security, and integrity. Any previous behaviors or events which have limited the Veterans ability to be approved for entry to the HCHV program must have clearly defined parameters of how the Veteran can be re-considered for admission. These parameters must be determined clinically appropriate by the VA Liaison. All Contractors must employ a harm reduction approach. Contractor will provide verbal and written notification that is readily accessible to all individuals including the VA liaison, upon request, of reasons for non-acceptance.

- 4.4. **Absences and Cancellation:** The Contractor shall notify the VA of unauthorized absences by a referred Veteran from the facility within 24 hrs. of Veterans absence. Should a Veteran absent himself/herself from the Contractor's facility in an *unscheduled* manner, payment for services for that Veteran shall be continued for a maximum period of 48 hours, provided the following conditions: (a) The provider must have evidence of active outreach to locate and reengage the Veteran and document the steps taken in the Veterans individual case file, treatment record, or service plan. (b) The provider may not fill the bed. (c) The Veteran must be discharged from the HCHV Contracted Residential Services (CRS) program if not located within 48 hours. (d) The Veteran cannot be enrolled in another HCHV or Grant and Per Diem (GPD) program during this time. Management of program dropout rate will be an element of quality assurance review of this program. The VA Liaison may authorize payment for *scheduled* absences up to 96 hours as long as the following conditions are met: (a) Be pre-planned, consistent with and support the Veteran's individual service plan (e.g., family reunification, short term medical, substance use disorder (SUD) or psychiatric treatment). (b) Have the reason documented in the individual Veteran's case file, treatment record, or service plan. (c) Not result in the bed being filled by the provider or Veteran enrolling in another program. (d) Not be for a break or vacation from treatment. (e) Not be used for extended educational or employment circumstances. (f) Not be used to create more than 4 consecutive days of absence.

- 4.4.1. **For Medical Respite and Geriatric HCHV Contract Emergency Residential Services (CERS) programs;** the per diem payment may be made for a period of up to 7 days to the medical respite or geriatric care provider if a Veteran requires inpatient medical or psychiatric treatment. The provider may not fill the bed during this time and should maintain contact with the HCHV VA Liaison and the Veteran if authorized.

- 4.4.1.1. **Ineligible Veterans:** When a Veteran is admitted to a HCHV CRS program and found to be ineligible for HCHV CRS program, the VA will pay for a maximum of 4 days from the day of admission to allow the provider and HCHV VA Liaison time to locate and arrange alternate placement.
- 4.4.2. VA reserves the right to remove any or all Veterans from the facility at any time, without additional cost, when it is determined to be in the best interest of the VA or the Veteran. This includes discharge from facility for ongoing, uncontrolled bed bug infestation or infection disease outbreak.
- 4.4.3. The Contractor shall notify the authorizing VA facility immediately when a medical emergency occurs that requires hospitalization of a referred Veteran. It is agreed that the Veteran will be admitted to the appropriate VA facility. When such admission is not feasible because of the nature of the emergency, it is agreed that hospitalization in a non-federal facility is acceptable. If hospitalization of a non-emergent nature is required, it is agreed that admission to the appropriate VA facility will be accomplished promptly. The Contractor will make arrangements for admission and support the Veteran with any transportation issues that may arise.
- 4.4.4. The Contractor shall notify the authorizing VA facility immediately of any incidents involving Veterans residing in the Contract Emergency Residential program. The Contractor shall notify the VA liaison by telephone or email during the hours of 8:00am and 4:30pm. For all incidents that occur after normal business hours, the Contractor should notify the HCHV Staff the following business day. The Contractor shall provide the HCHV case manager and the COR with a copy of the incident report within 24 business hours. The Contractor shall maintain a copy of the incident report in the Veteran's case record. Please see above in Basic Services for details regarding what constitutes Critical Incident Reporting.
- 4.5. **Basic Rights and Protection Against Discrimination:** Contractors must protect the rights and dignity of the individual or family served in all phases of service delivery. At a minimum, providers must afford each Veteran the following rights and protections. Clients must be permitted to exercise these rights without fear of reprisal.
- 4.5.1. Veterans are entitled to enjoy a safe and healthful environment in the program.
- 4.5.2. Veterans are entitled to be treated in a manner that respects their dignity, privacy, and individuality.
- 4.5.3. Veterans with disabilities are entitled to reasonable accommodations under fair housing laws when such accommodations are necessary because of their disability
- 4.5.4. Veterans are entitled to remain in the program and not be involuntarily removed without reasonable notice, good cause, and just procedures.
- 4.5.5. Veterans are entitled to reasonable privacy and confidential treatment of personal, social, financial, medical, mental, and behavioral health records, except as necessary to further treatment, information, and referral services and in compliance with the resident's consent to release information
- 4.5.6. Veterans are entitled to the full exercise of their civil, constitutional, and legal rights
- 4.5.7. Veterans will have on-going opportunities to voice opinions, to participate in program operation and programming, and to make suggestions regarding programming and rules.
- 4.5.8. Veterans' rights must be protected against all forms of discrimination, including those based on race, religious creed, color, national origin, ancestry, language, disability (physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation
- 4.5.9. Veterans will receive a written policy indicating that harassment of clients and staff based on race, religious creed, color, national origin, ancestry, language, disability

(physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation will not be condoned nor tolerated.

4.5.10. Contractors will develop a written policy for transgender clients that provides for safe, secure, and dignified case management as well as accommodate the special requirements needed for privacy. Transgender clients will have access to sleeping accommodations and bathroom facilities based on their *gender of identification, regardless of physical characteristics or gender conformity*. People who do not clearly identify as male or female should have access to whichever sleeping and/or bathroom accommodation helps them feel safest. Where there are single-use showers and bathrooms in the facility designated for residents, transgender residents will be told about them and welcome to use them but not required. It is the Contractors responsibility to promote a safe environment for transgender clients amongst the general population.

4.5.11. Programs must post these policies in a conspicuous place and in appropriate languages.

4.5.12. All policies and procedures will be in writing and subject to review by the VA

4.6. **Reasonable Accommodation:** Contractor must respect and reasonably accommodate personal and cultural differences associated with race, religious creed, color, national origin, ancestry, language, disability (physical or mental health), medical condition, marital status, familial status, age, gender, sexual preference, source of income, or political affiliation. At no point shall program access be denied because of an individual's disability. Information learned about the client's disability from the assessment will not be used as grounds for discharge or other punishment and will be used to guide client to appropriate services. Moreover, a program may not apply different rules to individuals because of their disabilities, unless the different rules are a result of granting a reasonable accommodation request made by an individual with a disability. The Contractor shall not ask questions about a client's disability, including the nature and severity of the disability or the treatment, symptoms, and medications related to the disability. A Veteran is not obligated to reveal that s/he has a disability prior to admission. Veterans with disabilities are entitled to reasonable accommodations. If a Veteran requests a change in a program's policy or procedure as an accommodation of his/her disability, the program should grant the accommodation when the accommodation is both reasonable and necessary because of the individual's disability. Such an accommodation request must be considered during any stage in the provision of a program, including at intake, during services, and during discharge proceedings. When a reasonable accommodation request is made, the program supervisor may request verification of the individual's disability. The Contractor is required to develop a written policy for their process for reasonable accommodations within 30 days of the award of this contract. This will include: 1) All requests for reasonable accommodation, 2) details of all steps taken to accommodate a Veteran's request for reasonable accommodation, and 3) any explanation for denial of services and/or accommodation, and 4) any appropriate referrals to Veterans who are denied accommodation. All explanations are to be written on formal letterhead from program management to the Veteran and Liaison at the time of denial for services or accommodation. Contractor will be aware that violation of the ADA, Section 504 of the Rehabilitation Act and Fair Housing Laws may result in stopping admissions and/or termination of contract for convenience.

4.7. **Procedures For Negative Discharge or Sanction:** Contractor must clarify rules in which behaviors constitute gross misconduct and are grounds for immediate discharge from the program and which would prompt a written warning if violated and potential discharge if violated repeatedly. There will be no permanent bans from the facility. A Veteran may be requested to participate in a cooling off period. (Please see Section C) There will be no “overnight outs.” Contractor will describe the formal appeal procedures through which clients may appeal program regulations, sanctions, or discharges. This information must be provided to clients in writing upon intake and must be clear and easily understandable by clients.

This information will include:

- Immediate contact with VA liaison
- Timely due process provisions which should include two warning notices for violations which do not result in immediate discharge prior to issuance of a discharge notice and an opportunity for a case conference after appropriate warning is issued to the client
- Notice of, and access to, formal appeal procedures
- Notice of the conditions or process for re-admission to the program.
- Reasonable efforts to provide an appropriate referral to another facility or appropriate level of care as needed
- This does not apply to Veterans who are discharged for danger to others.

5. **RELEASE OF MEDICAL INFORMATION:** The Contractor shall take reasonable measures to ensure Veteran residents’ privacy and confidentiality. The contractor must obtain a signed ROI within 24 hours of admission. ROI’s may not be dated over one year from the date of Veteran’s signature on the ROI. ROI’s must be updated at each admission and/or annually if applicable Veteran resident’s demographics shall be treated as privileged information. The Contractor shall only release medical information obtained during the course of this contract to VA employees involved in the care and/or treatment of the Veteran residents. The Contractor personnel shall make sure no Veteran resident information of any type is given to outside parties, agencies, or organizations of any type without the expressed written consent of the Veteran resident and the VA and only in capacities directly related to the provision of the services contracted in this instrument. Lists, names, and/or social security numbers of Veteran residents shall not be disclosed or revealed in any way, for any use outside the Contractor’s facility or without prior express written permission of the Contracting Officer or the Veteran resident. That only the minimum necessary Veteran resident information is used to provide appropriate service to the correct Veteran resident. Transcribed reports containing personal identifiers, when not used as an official document, must be shredded by the Contractor. The Contractor is subject to the same penalties and liabilities for unauthorized disclosures of such records as VA personnel as prescribed in VAAR Clause 852.273-75 listed in Section C of this document with attention given to VA Handbook 6500.6, Contract Security Requirements for Unclassified Information Technology Resources, found at this [link](#).

6. **STAFF/QUALIFICATIONS/ACCREDITATION/LICENSING**

6.1. **Personnel:** The Contractor shall assign personnel that by education, training, and when required certification or licensure, to be qualified to provide the Basic Services and Supplemental Services required by this PWS. The Contractor will employ sufficient personnel to carry out the policies, contract responsibilities, and the program for the facility.

6.1.1. This contract requires a dedicated, licensed medical case manager with credentials

such as LVN, RN, and/or BSN. The medical case manager must be full-time and work during normal business hours Monday through Friday. The medical provider must be identified at start of contract and must have proof of unencumbered license according to their licensing board. The licensed medical case manager will be comfortable working with Veterans who may have co-morbidities which include those diagnosed with a severe mental illness and/or substance use disorder.

- 6.1.2. Case management staff must have appropriate and commensurate coverage when on leave (e.g., holidays, sick leave, family care, etc.). Staff identified by the Contractor to provide temporary leave coverage will be expected to cover the same schedule as the primary medical case manager and have the same credentials unless a waiver is granted by the VA Liaison or Designee. The VA Liaison or Designee is expected to be notified of cross coverage if the primary Contractor medical case manager is absent prior to the leave and/or within 24 hours of unexpected absences. If the leave is covered by both a case manager and an on-call medical provider, the medical provider must be able to be on site within 15 minutes.
- 6.1.3. There must be, at a minimum, one staff member on duty, on the premises and residing or stationed at the facility where the Veterans resides that is available for emergencies 24 hours a day, 7 days a week. This staff member will be expected to be made aware of their coverage duties and trained in appropriate Contractor emergency protocols, CPR/First Aid and HIPPA certified, and an understanding of basic de-escalation techniques. Appropriate security measures will be in place to ensure facilities with detached residences/rooms will have adequate oversight by the facility for each separate location where Veterans reside. Veterans will be provided the staff member information on intake and informed on whom to call in case of emergencies. If there is not a full-time medical case manager for night and weekend shifts, then an on-call medical case manager may be used with the same requirement, LVN/RN or BSN licensure, as the daytime medical case manager. The on-call medical case manager must be able to arrive to the shelter within 15 minutes Contractor call. All night, day and weekend staff that will be providing Veteran care will be required to be, at minimum, certified medical technicians. A list of these staff members and their schedules will be requested at start of contract. The night and weekend staff may be expected to assist the LVN with appropriate medication monitoring duties if determined to be clinically necessary.
- 6.1.4. There is an expectation that there will be an administrative/clinical Designee that will be available for consultation for emergencies 24 hours a day, 7 days a week.
- 6.1.5. Contractors will not assign or hire Staff to perform the clinical and medical duties outlined in this contract who are currently enrolled in a VA Homeless Program or who have ended participation in a VA Homeless Program within the past two years.
- 6.1.6. Contractors must have a policy prohibiting staff from establishing sexual relationships with program clients.
- 6.1.7. Contractor staff may not be living in the same rooms as participants in the program.
- 6.1.8. The Contractor must identify each person functioning as “Key Personnel” under this contract and provide to the VA a description of the services to be provided by such person, together with a resume summarizing that person’s relevant skills and experience. **Key personnel for purposes of this contract are the medical case manager, case manager supervisor and the manager for night and weekends and name of, or agency to be used, the on call LVN.**
- 6.1.9. During the first ninety (90) calendar days of contract performance, the Contractor shall make NO substitutions of key personnel unless the substitution is necessitated by illness, death, or termination of employment. Within 14 days after substitutions necessitated by situations described above, the Contractor shall provide resumes for

the substitute key personnel. For substitutions proposed by the Contractor after the initial 90 calendar day period, the Contractor shall provide resumes for the substitute personnel, together with any other additional information requested by the Contracting Officer, **at least 15 days before the substitution is to occur**. At this point the VA shall review and give feedback on personnel. The Contracting Officer shall notify the Contractor within fifteen (15) calendar days after receipt of all required information if the VA refuses to accept the substitute key personnel. The VA reserves the right to refuse or revoke acceptance of key personnel if personal or professional conduct, or lack of required skills or experience, jeopardizes patient care or interferes with the regular and ordinary operation of the facility. Temporary substitutions of key personnel shall be permitted in accordance with the Contractor's contingency plan. The Contractor's contingency plan to be utilized if personnel leave Contractor's employment or are unable to continue performance in accordance with the terms and conditions of the resulting contract must be submitted as part of the Contractor's offer.

6.2. **Staff Training:** Contractor will always provide at least one staff person on-site who has had training and orientation on the following topics. Staff shall receive and document—at a minimum—one-time training per year on these subjects.

- CPR
- First Aid
- Crisis intervention
- Cultural sensitivity
- Sexual harassment
- Sensitivity to wider issues of homelessness
- Universal Precautions (disease transmission prevention)
- Adult abuse/neglect reporting laws
- Harm Reduction philosophy
- Suicide Prevention
- Medication management
- State and Federal Fair Housing Law and ADA Requirements
- Conflict resolution
- Customer Service
- Professional Boundaries
- Countertransference
- Motivational Interviewing and Stages of Change
- Severe Mental Illness and Psychopathology
- De-escalation techniques
- Search and Seizure/Probable Cause in the Shelter Setting
- Conservatorship
- Grave Disability Criteria

6.3. **Contractor Staff Conduct/Complaints Handling:** Contractor personnel shall be expected to treat referred Veterans with dignity and respect and abides by standards of conduct mirroring those prescribed by current federal personnel regulations. The Contractor shall comply with the VA Patient's Bill of Rights as set forth in 38 CFR 17.34a (copy available upon request).

6.3.1. The VA reserves the right to exclude Contractor staff members from providing services to Veterans under this contract based on breaches of conduct, including conduct that jeopardizes patient care or interferes with the regular and ordinary

operation of the facility. Breaches of conduct include intoxication or debilitation resulting from drug use, theft, patient abuse, dereliction, or negligence in performing directed tasks, or other conduct resulting in formal complaints by Veterans or other Staff members to designated Government representatives. The Contractor and Contracting Officer's Representative shall deal with issues raised concerning Contractor personnel conduct. The Contracting Officer shall be the final arbiter on questions of acceptability and in validating complaints.

7. **DESCRIPTION AND STANDARDS OF FACILITIES**

- 7.1. **Facility:** It is the responsibility of the Contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the Contractor for such expenses. The contract facility must:
- 7.1.1. Pass VA NFPA inspection within 60 days of the award of this contract if they have not been previously awarded a contract. Contract will be terminated for convenience if facility does not pass NFPA inspection within 60 days.
 - 7.1.2. Have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
 - 7.1.3. Be in compliance with existing standards of the National Fire Protection Act (NFPA), State and local safety codes, and/or State health and sanitation codes.
 - 7.1.4. Where applicable, be licensed under State or local authority.
 - 7.1.5. Where applicable, be accredited by the State.
 - 7.1.6. Be equipped with operational air conditioning /heating systems
 - 7.1.7. Be kept clean free of dirt, grime, mold, or other hazardous substances and damaged noticeably detract from the overall appearance.
 - 7.1.8. Be equipped with first aid equipment and an evacuation plan in case of emergency.
 - 7.1.9. Have windows and doors that can be opened and closed in accordance with manufacturer standards.
 - 7.1.10. Have an aggressive on-going plan to address bed bug infestation and infection control. ***This policy must be a part of your written response to this solicitation.*** On-going bed bug infestation will be grounds for immediate discharge of Veterans from the facility.
 - 7.1.11. Meet fire safety requirements, as follows:
 - 7.1.11.1. The building must meet the requirements of the applicable residential occupancy chapters of the current version of NFPA 101, National Fire Protection Association's Life Safety Code. Any equivalencies or variances must be approved by VAPAHCS Director
 - 7.1.11.2. All residents in the facility must be mentally and physically capable of leaving the building, unaided, in the event of an emergency
 - 7.1.11.3. Fire exit drills must be held at least quarterly. Residents must be instructed in evacuation procedures. A written fire plan for evacuation in the event of fire shall be developed and reviewed annually. The plan shall outline the duties, responsibilities, and actions to be taken by the staff and residents in the event of a fire emergency. Plan shall be implemented during fire exit drills.
 - 7.1.11.4. A written policy regarding tobacco smoking in the facility shall be

established and enforced.

7.1.11.5. NFPA 10 Portable fire extinguishers shall be installed at the facility and NFPA will be used as guidance in selection and location requirements of fire extinguishers.

7.1.11.6. Requirements for fire protection equipment and systems shall be in accordance with NFPA 101. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested, and maintained in accordance with the applicable NFPA fire codes and the results documented.

7.1.11.7. The annual inspection by a VA team required by paragraph 7 shall include a fire and safety inspection conducted at the facility unless a review of past Department of Veterans Affairs inspections or inspections made by the local authorities indicates that a fire and safety inspection would not be necessary, in which case the fire and safety inspection may be waived by the VA.

7.2. **Inspection Of Facility And Program:** Prior to the award of a contract and annually during the contract term, a multidisciplinary VA team consisting of a social worker, dietitian, a representative of the VA Police, and a Facilities Management Safety Officer, Nursing and/or other subject matter experts as determined necessary by the medical center director or HCHV Coordinator shall conduct a survey of the Contractor's facilities to be used to provide Veterans food, shelter, and therapeutic services to assure the facility provides quality care in a safe environment. Inspections may also be carried out at such other times as deemed necessary by the Department of Veterans Affairs.

7.2.1. The Contractor will be advised of the findings of the inspection team. If deficiencies are noted during any inspection, the Contractor will be given a reasonable time (90 days) to take corrective action and to notify the Contracting Officer that the corrections have been made. A contract will not be awarded until noted deficiencies have been eliminated. Failure by the Contractor to take corrective action within a reasonable time (90 days) will be reported to the VA Contracting Officer. If corrections are not made to the satisfaction of the VA, the Contracting Officer will consult with the appropriate officials so that suitable arrangements can be made to discontinue plans to award a contract, or to discharge or transfer patients and to terminate the existing contract, as appropriate. The inspection of the Contractor facilities will include inspection for conformity to the current Life Safety Code as described in paragraph 5, and will also include the following:

7.2.1.1. General observation of residents to determine if they maintain an acceptable level of personal hygiene and grooming.

7.2.1.2. Assessment of whether the facility meets applicable fire, safety, and sanitation standards.

7.2.1.3. Determining whether the facility is in attractive surroundings conducive to social interaction and the fullest development of the resident's rehabilitative potential.

7.2.1.4. Observation of facility operations to see if appropriate organized activity programs are available during waking hours (including evenings) and degree to

which a high level of activity is observed in the facility, such as individual professional counseling, physical activities, assistance with health and personal hygiene.

7.2.1.5. Seeking evidence of facility-community interaction, demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services, etc.

7.2.1.6. Observation of staff behavior and interaction with residents to determine if they convey an attitude of genuine concern and caring.

7.2.1.7. Inspecting the types of meals and other nutrition provided to residents to see if appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and if nutritious snacks between meals and bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated.

7.2.1.8. Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.

7.2.1.9. All Department of Veterans Affairs reports of inspection of residential facilities furnishing treatment and rehabilitation services to eligible Veterans shall, to the extent possible, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

7.3. **Emergency procedures:** Contractor will train all staff on emergency procedures and have written protocols that are posted to guide staff response to crises including, but not limited to, physical injury, resident suicide attempts, overdoses, and domestic or other violence.

7.3.1. Contractors will always have *at least one designated paid staff on site* who has had training and had orientation on emergency procedures.

7.3.2. Contractor will promptly and appropriately respond to the medical/psychiatric problems of clients and staff.

7.3.3. There will be first aid equipment and supplies for medical emergencies available at all times. These supplies must be checked regularly to ensure they are up to date and their location in the facility must be clearly marked.

7.3.4. Contractor will have all emergency contact numbers posted.

7.3.5. Universal precaution practices are used by Contractor to prevent transmission of diseases and are implemented under the presumption that blood and body fluids from any source are to be considered potentially infectious. Supplies necessary for maintaining universal precautions, such as sharps containers, must be available.

8. **BILLING:** Unless specifically excluded in this contract, the per diem rate established will include the services listed in this document and will also include all services or supplies normally provided other residents by the facility without extra charge. Payments made by the VA under this contract shall constitute the TOTAL cost of care and housing of the homeless Veterans. Payment is authorized starting from the first day of admission and does not include the day of discharge. Absences of the Veteran from the facility will not be reimbursed if reported later than 48 business hours after the absence. The Contractor will submit billing to Austin Payment Center by the 7th of every month. Failure to make timely invoice submissions will be cause for a request for a corrective action and if invoicing issues continue to arise it will be grounds for termination for convenience.

- 8.1. The Contractor shall be expected to utilize nightly sign in logs for the purpose of verifying a Veteran's attendance in the program on a daily basis. These logs are to be submitted to the VA Liaison monthly, along with invoices, to ensure accuracy of billing.
- 8.2. All scheduled and unscheduled absences will be clearly documented on the monthly attendance log including reason for any scheduled absence (e.g. medical needs). Payment will not be received for any unscheduled absences over 48 hours nor scheduled absences over 96 hours"). For Medical Respite and Geriatric contracts only, the per diem payment may be made for a period of up to 7 days if a Veteran requires inpatient medical or psychiatric treatment (as defined in the previous section titled "absences and cancellation). However, Veteran does not have to be formally discharged and may remain in the program for up to 14 days for scheduled absences. For unscheduled absences and Veterans found to be ineligible for the program after admission, there must be a discharge after 4 days. No payment will be made without the daily sign-in logs to be verified by VA Staff. The contractor must ensure their documentation clearly illustrates that the Veteran has in fact been present in a 24- hour span. The contractor cannot bill for a Veteran until formally admitted into the VA's HCHV program. A Veteran can only be billed up to seven calendar days prior to meeting a VA liaison for intake. Exceptions can be made on a case by case basis with approval from the VA Liaison (or Designee). Arranging the meeting between the Liaison and the Veteran is the contractor's responsibility.

9. **QUALITY ASSURANCE AND PERFORMANCE MONITORING.** Contractor will be held to the same standard as VA personnel through quality assurance and performance monitoring to ensure compliance under this contract. These procedures will include time and attendance logs verification, monitoring of the Computerized Patient Record System (CPRS) records or other appropriated records called for under this contract received by the VA. The Quality Assurance and Performance Monitoring will be governed by the appointed COR who will be responsible for verifying compliance with the terms and conditions of the contract. After the contract is awarded, any incidents as evidenced by the monitoring procedures will be forwarded immediately to the Contracting Officer. In addition, the COR will forward the summary evaluation of the Contractor performance to the Contracting Officer once performance ends.

- 9.1. **QUARTERLY REPORTING:** Upon request the Contractor shall provide the Liaison written report of program activities on a Quarterly basis. The report may contain the following information:

- a. Total Number of Veterans Served
- b. Data from QASP compliance tool
- c. Program Completion Rates
- d. Outcomes (e.g. # or % of veterans discharged to *independent* housing, increased income/benefits, occupancy, etc.)
- e. Other pertinent information, such as: quality improvement projects, changes in staffing or business practices, systems, or resource concerns, etc.
- f. Reasons for negative discharges

These reports shall be maintained by the Liaison in the contract administrative file, and all necessary reports shall be submitted to the contracting office in a quarterly basis as a

part of the annual inspection package. VA staff will have access to all charts upon request for review.

10. **CORRECTIVE ACTION RESPONSE TO VIOLATION (CARV):** Contractor will be subject to the CARV. Depending on the nature of the violation Contractor will submit a corrective action plan to the COR (or Designee) within a timeframe *designated* by the COR (or Designee) and based on severity of violation (e.g., Safety issues). The program will be monitored for performance. Failure to perform duties herein will have the contractor be subject to contract termination.
11. **COVID 19 REQUIREMENTS:** The contractor must have provisions in place or will have provision in place before the start of the contract based on guidance provided by the county and local VA Providers. The contractor must abide by whatever social distancing, shelter-in-place, isolation, mask, and/or vaccination mandates/recommendation that are given by the county and VA providers. Additional provisions that go above and beyond the county guidelines and that may affect admissions must be discussed and agreed up on with the COR/Program manager.

The contractor will provide additional PPE supplies to the employees and the residents of the facility to include N95 masks, hand sanitizer, hand soap, etc. The contractor is responsible for the daily professional cleaning of all bedrooms and common areas, including the deep cleaning of all carpets and floors.

ADDITIONAL CONTRACT REQUIREMENTS

- I. **SUPPLEMENTAL SERVICES:** All services shall be provided at no additional charge to the stated daily fixed fee, except for those services specifically stated below to be Supplemental Services.

The C&A requirements do not apply, and a Security Accreditation Package is not required.

